

Competency Questions Using GPT 5.5 Thinking

CQ1: Can FASTO distinguish hypoglycemia severity levels, especially level 2 and level 3 events, and use those levels to identify when a diabetes treatment plan should be reevaluated?

CQ2: For patients taking insulin or otherwise at high risk for hypoglycemia, can FASTO represent that glucagon should be prescribed and that family members, caregivers, school personnel, or other support persons should be trained to administer it?

CQ3: Can FASTO classify rescue glucagon formulations by preparation burden and route, such as non-reconstituted glucagon nasal powder, prefilled pen or syringe, dasiglucagon, and powdered glucagon requiring diluent?

CQ4: Can FASTO represent the initial treatment protocol for conscious hypoglycemia: fast-acting glucose or carbohydrate, avoidance of high-fat or high-protein foods for initial treatment, glucose recheck after approximately 15 minutes, and repeated treatment if hypoglycemia persists?

CQ5: Can FASTO represent impaired awareness of hypoglycemia, including screening by validated single-question or multi-item tools such as Pedersen-Bjergaard, Gold, Clarke, or HypoA-Q, and link the result to hypoglycemia risk?

CQ6: Can FASTO represent fear of hypoglycemia as a psychosocial barrier to achieving glycemic targets and well-being, including a screening question and association with anxiety, depression, hypoglycemia confidence, and HbA1c outcomes?

CQ7: Can FASTO represent continuous glucose monitoring metrics such as time in range, time below range, time above range, glucose management indicator, coefficient of variation, and mean glucose, and relate those metrics to individualized glycemic goals?

CQ8: Can FASTO classify diabetes technology into BGM, CGM, insulin pump or continuous subcutaneous insulin infusion, connected insulin pens, and automated insulin delivery systems, including the relation by which AID uses CGM-informed algorithms to modulate insulin delivery?

CQ9: Can FASTO represent advanced hybrid closed-loop system use and its real-world outcome profile, including TIR/TBR changes, severe hypoglycemia, ketoacidosis, skin/adhesive problems, practical barriers, and treatment satisfaction?

CQ10: Can FASTO model mealtime insulin adjustment using carbohydrate intake together with fat and protein intake, concurrent glycemia, glycemic trends, sick-day management, and anticipated physical activity?

CQ11: Can FASTO distinguish insulin-delivery treatment modalities for adults with type 1 diabetes, including continuous subcutaneous insulin infusion, multiple daily injections of basal and prandial insulin, inhaled insulin, and the distinction between insulin analogs and human insulin?

CQ12: Can FASTO determine whether an insulin treatment plan has been reevaluated within a recommended interval, such as every 3 to 6 months, and record the factors used to adjust the plan to individualized glycemic goals?

CQ13: For children and adolescents with type 1 diabetes whose diabetes is partly or wholly managed by someone else, can FASTO link device selection, CGM education, glucagon preparedness, and learning plans to caregiver skills and preferences?

CQ14: Can FASTO represent age- and development-sensitive glycemic assessment frequency, including more frequent assessment during rapid growth and development in children and adolescents and individualized targets for older adults?

CQ15: Can FASTO represent transition planning from pediatric to adult diabetes care, including readiness, transfer timing, and continuity of diabetes-management responsibilities?

CQ16: For older adults with diabetes, can FASTO represent geriatric syndromes and related risks—such as cognitive impairment, depression, falls, frailty, persistent pain, urinary incontinence, hypoglycemia, and polypharmacy—and use them to individualize goals and therapeutic approaches?

CQ17: Can FASTO represent social determinants of health and treatment-burden factors—including financial considerations, food insecurity, support systems, individual values, and preferences—as inputs to treatment decisions and hypoglycemia-risk mitigation?

CQ18: Can FASTO represent dietary patterns and evidence strength for type 1 diabetes management—such as high-fiber diets and carbohydrate-restricted diets—and link them to outcomes including HbA1c, TIR, coefficient of variation, hypoglycemia, insulin dose, and anthropometric measures?